

Prescribing Information
CAVSAMIR (Sevelamer Carbonate Tablets 800 mg)

a) Name and Strength of Active Substance & Excipients

Each film coated tablet contains:
Sevelamer carbonate 800mg
Excipients: Mannitol, Crospovidone Type B (XL-10), Hydroxypropyl Cellulose (Klucel LF), Colloidal Silicon Dioxide, Zinc Stearate, Opadry AMB Translucent (OY-B-29000), Purified water, Opacode Black S-1-17823 & Isopropyl alcohol.

b) Product Description:

White to off –white , oval, film coated tablets plain on one side and imprinted 'R 789' on other side.

c) Pharmacodynamic/ pharmacokinetic properties

Pharmacodynamic properties

Sevelamer carbonate contains sevelamer, a non-absorbed phosphate binding crosslinked polymer, free of metal and calcium. Sevelamer contains multiple amines separated by one carbon from the polymer backbone which become protonated in the stomach. These protonated amines bind negatively charged ions such as dietary phosphate in the intestine. By binding phosphate in the gastrointestinal tract and decreasing absorption, sevelamer lowers the phosphorus concentration in the serum. Regular monitoring of serum phosphorus levels is always necessary during phosphate binder administration.

Pharmacokinetic properties

Pharmacokinetic studies have not been carried out with sevelamer carbonate. Sevelamer hydrochloride, which contains the same active moiety as sevelamer carbonate, is not absorbed from the gastrointestinal tract, as confirmed by an absorption study in healthy volunteers.

d) Indications

CAVSAMIR is indicated for the control of hyperphosphataemia in adult patients receiving haemodialysis or peritoneal dialysis.

CAVSAMIR is also indicated for the control of hyperphosphataemia in adult patients with chronic kidney disease not on dialysis with serum phosphorus ≥ 1.78 mmol/l.

CAVSAMIR should be used within the context of a multiple therapeutic approach, which could include calcium supplement, 1,25-dihydroxy Vitamin D3 or one of its analogues to control the development of renal bone disease.

e) Recommended dosage

Starting Dose: One or two 800-mg tab 3 times daily with meals.

Because of the rapid disintegration of the carbonate salt tablet and its rapid reaction with the hydrochloric acid in the stomach, the dosing of sevelamer is anticipated to be similar to that of the hydrochloride salt.

Patients Not Taking a Phosphate Binder: Recommended Starting Dose: 800-1600 mg, which can be administered as 1 or 2 sevelamer 800-mg tablets, with meals based on serum phosphorus level. The table as follows provides the recommended starting doses of sevelamer for patients not taking a phosphate binder. (See Table 1)

Table:1 Starting dose for patients not taking a phosphate binder	
Serum Phosphorus	Cavsamir 800mg
>1.78 and < 2.42 mmol/l	1 tablet 3 times daily with meals
≥2.42 and <2.91 mmol/L	2 tablets 3 times daily with meals
≥2.91 mmol/L	2 tablets 3 times daily with meals

Patients Switching From Sevelamer Hydrochloride: For patients switching from sevelamer hydrochloride, sevelamer carbonate should be prescribed on a gram per gram basis. Further titration to the desired phosphate levels may be necessary. The highest daily dose of sevelamer carbonate studied was 14 g in chronic kidney disease (CKD) patients on dialysis.

Patients Switching From Calcium Acetate. In a study in 84 CKD patients on hemodialysis, a similar reduction in serum phosphorus was seen with equivalent doses (approximately mg for mg) of sevelamer hydrochloride and calcium acetate. Table 2 gives recommended starting doses of CAVSAMIR based on a patient's current calcium acetate dose.

Table 2. Starting Dose for Dialysis Patients Switching From Calcium Acetate to CAVSAMIR

Calcium Acetate 667 mg (Tablets per meal)	CAVSAMIR 800 mg (Tablets per meal)
1 tablet	1 tablet
2 tablets	2 tablets
3 tablets	3 tablets

Dose Titration for All Patients Taking sevelamer: The dose should be increased or decreased by 1 tab/meal at 2-week intervals, as necessary, with the goal of controlling serum phosphorus within the target range of 1.13-1.78 mmol/L.

Patients taking Sevelamer should adhere to their prescribed diets.

Paediatric population

The safety and efficacy of sevelamer carbonate has not been established in pediatric patients. Sevelamer is not recommended for use in children below 18 years.

For paediatric patients the oral suspension should be administered, as tablet formulations are not appropriate for this population.

f) Route of administration

For oral use.

Tablets should be swallowed intact and should not be crushed, chewed, or broken into pieces prior to administration.

g) Contraindications

- Hypersensitivity to the active substance or to any of the excipients
- Hypophosphataemia
- Bowel obstruction.

h)Special warnings and precautions for use

Sevelamer is currently not recommended for use in adult patients with chronic kidney disease not on dialysis with serum phosphorus < 1.78 mmol/l.

Caution should be exercised when Sevelamer is used in patients with the following disorders:

- Dysphagia
- Swallowing disorders
- Severe gastrointestinal motility disorders including untreated or severe constipation or major gastrointestinal tract surgery. Use with caution in patients with these gastrointestinal disorders. Monitor serum chemistries: Bicarbonate and chloride levels should be monitored.

Reductions in Vitamins A, D, E, K (clotting factors) and Folic Acid Levels:

Patients with CKD may develop low levels of fat-soluble vitamins A, D, E and K, depending on dietary intake and the severity of their disease. It cannot be excluded that Sevelamer can bind fat-soluble vitamins contained in ingested food. In patients not taking supplemental vitamins but on sevelamer, serum vitamin A, D, E and K status should be assessed regularly. It is recommended that vitamin supplements be given if necessary. It is recommended that CKD patients not on dialysis are given vitamin D supplements (approximately 400 IU of native vitamin D daily) which can be part of a multivitamin preparation to be taken apart from their dose of Sevelamer. In patients undergoing peritoneal dialysis additional monitoring of fat-soluble vitamins and folic acid is recommended.

i) Interaction with other medicinal products and other forms of interaction

There are no empirical data on avoiding drug interactions between sevelamer and most concomitant oral drugs. For oral medication where a reduction in the bioavailability of that medication would have a clinically significant effect on its safety or efficacy (e.g., cyclosporine, tacrolimus, levothyroxine), consider separation of the timing of the administration of the two drugs. The duration of separation depends upon the absorption characteristics of the medication concomitantly administered, such as the time to reach peak systemic levels and whether the drug is an immediate release or an extended release product. Where possible consider monitoring clinical responses and/or blood levels of concomitant drugs that have a narrow therapeutic range. Sevelamer carbonate did not alter the pharmacokinetics when administered concomitantly in the following drugs digoxin, enalapril, iron, metoprolol and warfarin.

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The anti-arrhythmic and anti-seizure medical product should be taken at least one hour before or three hours after sevelamer, and blood monitoring can be considered.

Very rare cases of increased phosphate levels have been reported in patients taking proton pump inhibitors co-administered with sevelamer carbonate. Caution should be exercised when prescribing PPI (omeprazole, pantoprazole, or lansoprazole) to patients concomitantly treated with sevelamer. The phosphate serum level should be monitored and the sevelamer carbonate dosage adjusted consequently.

Oral drugs that have demonstrated interaction with sevelamer and are to be dosed separately from sevelamer

Ciprofloxacin: Dosing Recommendations - Take at least 2 hours before or 6 hours after sevelamer

Mycophenolate mofetil: Dosing Recommendations- Take at least 2 hours before sevelamer

j) Fertility and lactation

Pregnancy

There are no or limited amount of data from the use of sevelamer in pregnant women. Sevelamer has been shown to reduce the absorption of several vitamins including folic acid. The potential risk to humans is unknown. Sevelamer carbonate should only be given to pregnant women if clearly needed and after a careful risk/benefit analysis has been conducted for both the mother and the foetus.

Breast-feeding

It is unknown whether sevelamer/metabolites are excreted in human milk. The non-absorbed nature of sevelamer indicates that excretion of sevelamer in breast milk is unlikely. A decision on whether to continue/discontinue breast-feeding or to continue/discontinue therapy with sevelamer carbonate should be made taking into account the benefit of breast-feeding to the child and the benefit of sevelamer carbonate therapy to the woman.

Fertility

There are no data from the effect of sevelamer on fertility in humans. It has been shown that sevelamer did not impair fertility in male or female rats at exposures at a human equivalent dose 2 times the maximum dose of 13 g/day, based on a comparison of relative BSA.

k) Undesirable effects

Summary of the safety profile

The most frequently occurring adverse reactions were all in the gastrointestinal disorders system organ class. Most of these adverse reactions were mild to moderate in intensity.

Tabulated list of adverse reactions

Adverse reactions that occurred are listed by frequency in the table below. The reporting rate is classified as very common, common, uncommon, rare, very rare, not known.

MedDRA System Organ Class	Very common	Common	Very rare	Not known
Immune system disorders			Hypersensitivity*	
Gastrointestinal disorders	Nausea, vomiting, upper abdominal pain, constipation	Diarrhoea, dyspepsia, flatulence, abdominal pain		Intestinal obstruction, ileus/subileus, intestinal perforation ¹ , gastrointestinal haemorrhage* ¹ , intestinal ulceration* ¹ , gastrointestinal necrosis* ¹ , colitis* ¹ ,
				Intestinal mass* ¹
Skin and subcutaneous tissue disorders				Pruritus, rash
Investigations				Crystal deposit intestine* ¹

*Post-marketing experience

¹See inflammatory gastrointestinal disorders warning in section Special warnings and precautions for use.

Paediatric population

In general, the safety profile for children and adolescents (6 to 18 years of age) is similar to the safety profile for adults.

l) Overdose and Treatment

In CKD patients on dialysis, the maximum average daily dose was 14grams of sevelamer carbonate in a single daily dose. There are no reports of overdosage with sevelamer carbonate or sevelamer hydrochloride in patients. Since sevelamer is not absorbed, the risk of systemic toxicity is low.

m) Effects on Ability to Drive and Use Machines

Sevelamer has no or negligible influence on the ability to drive and use machines

n) Storage Conditions

Store below 30°C. Dispense in a tight container. Protect from moisture

o) Shelf Life

36 months

p) Dosage forms and packaging available

- 30's Count Tablets HDPE Bottle Packed in Outer carton With Pack insert.
- 270's Count Tablets HDPE Bottle with Pack insert.

q) Name and address of manufacturer/ product registration holder

Manufactured By

Dr. Reddy's Laboratories Limited
FTO SEZ - Process Unit 1,
Survey No.'s 57 to 59, 60, 62 & 72,
Sector No's. 9 to 14 & 17 to 20,
Devunipalavalasa (V), Ranasthalam (M),
Srikakulam District - 532409,
Andhra Pradesh, India.

Product Registration Holder

GoodScience Sdn Bhd
No. 7, Jalan KPK 4/3,
Kawasan Perindustrian Kundang,
Kundang Jaya, 48020 Rawang,
Selangor, Malaysia.

r) Date of revision of PI

20 January 2026